



Diagnostic Audit — Fieldwork Kit

Interview guides · observation checklist · fieldwork plan

Collaborative posture throughout: we are a partner in the findings, not an inspector. Open each interview with reassurance, listen more than we talk, and never make frontline staff feel blamed for the system's gaps.

1. Interview guides (30–45 min each)

Common opener: “Thank you for the time. This is not about individuals — it's about how the place works and how to make it stronger. Nothing you say is attributed to you by name.”

Owners / board (Kabir, Mrs Abisodun)

- What does success for Haven in 12 months look like? What worries you most?
- Walk me through the money: a good month vs a bad month, and why.
- Where do you feel cash disappears? What have you tried?
- How are decisions made day to day, and who runs the facility operationally?
- Appetite for a senior operations hire, and for the incentive / commission redesign?

Clinical lead / Medical (Dr Shakirah)

- Talk me through clinical governance: protocols, incident review, M&M.
- Where do you feel clinical risk concentrates?
- How ready is NICU to safely take more admissions? What would you need first?
- How are clinical standards set, taught and checked? What slips under pressure?

Medical Director / Neonatology (Dr Odedina)

- NICU case mix, admission criteria, staffing model, equipment readiness.
- What does safe scaling of NICU require — people, kit, protocols, cover?
- Where are the near-misses you worry about?

Nursing lead / Matron

- Walk me through a shift, start to finish. Where does it break down?
- Which routines are meant to happen every shift, and which actually do?
- Staffing levels vs workload — where is it thin, and when?
- How is performance recognised or addressed on your team?

Pharmacy lead

- How is stock ordered, received, stored, dispensed and counted?
- How often do you stock out, and of what? What drives it?
- How is pharmacy margin worked out? Where is it lost?
- What would vendor-managed inventory change for you?

Front desk / Customer service

- Walk me through a patient from arrival to billing.
- Where do patients wait, get frustrated, or fall through the cracks?
- Booking, deposits, HMO verification — how does it actually work?

Accounts / Admin

- How are the numbers produced? What system, what reconciliations, what's manual?
- Receivables: how are HMO claims tracked, chased and reconciled?
- What do you not trust in the current reporting, and why?

2. On-site observation checklist

Emergency readiness



- ☐ Crash cart located, sealed, contents vs a standard list, expiry dates
- ☐ Evidence of a shift-level check routine (log, sign-off)
- ☐ Emergency drugs accessible; oxygen, suction, resuscitaire (NICU) functional
- ☐ Staff can state where things are and what to do

Clinical & nursing routines

- ☐ Handover observed — structured? information lost?
- ☐ Drug administration and charting practice
- ☐ Hand hygiene / IPC in practice; documentation completeness on live charts

Patient flow

- ☐ Registration → triage → consult → admission/discharge → billing, timed
- ☐ Bottlenecks, queues, rework; booking system used as intended?

Pharmacy & stock

- ☐ Storage conditions, organisation, expiry management
- ☐ Physical spot-count vs system on a sample of lines; reorder discipline in practice

Facility & environment

- ☐ Bed configuration vs records; NICU environment
- ☐ Signage, cleanliness, safety hazards; equipment vs asset register on a sample

3. Fieldwork plan

Week	On-site / remote	Focus
1	Kick-off + remote	Nominate point person; issue data request; launch staff survey; crash-cart standard live; begin Leadway/NEM receivables push; start reconciling management accounts
2	On site	Leadership + frontline interviews; observation across shifts; SOP walk-throughs; NICU readiness
3	On site + remote	Finance & working capital; procurement & inventory spot-counts; HMO economics; reporting-integrity reconciliation; close survey
4	Remote + board	Synthesis; prioritised, costed recommendations; board presentation of the two deliverables

Deliverables: (1) Operational & Clinical Governance Audit Report; (2) Working Capital & Receivables Recovery Plan.